Cranial Nerve Examination — OSCE Checklist

Deakin MD2 · Neuro · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are usually verbalised in the OSCE ("I would also…"). Compare* **left vs right** *throughout and narrate aloud.*

1. Introduction (WIPER)

☐	**W**ash hands / hand hygiene
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**P**osition: seated, eye level, ~1 m apart
☐	Ask about **pain** before starting
☐	Equipment ready: pen torch, Snellen, red pin, ophthalmoscope, cotton wool, neurotip, 512 Hz fork, tongue depressor

2. General inspection (end of bed, 10–15 s)

	Look for	Significance
☐	Well or unwell	Distress, drowsiness, airway
☐	Bedside clues	Walking aids, NG tube, thickened fluids
☐	Head shape	Craniotomy scar, shunt, Paget's
☐	Ptosis / proptosis, pupils, gaze	CN III, Horner's, thyroid eye disease
☐	Facial asymmetry	Droop, flat nasolabial fold (CN VII / stroke)
☐	Scars, cachexia, rashes, skin lesions	Parotid/neck scars, ear vesicles (Ramsay Hunt)
☐	Hearing aids, glasses	Baseline sensory deficit
☐	Gait and posture	Ataxia, hemiplegic gait

3. Cranial nerves I–XII

	Step	What to do / say	Abnormal
CN I — Olfactory			
☐	Smell*	"Any change in smell or taste?" Formal test one nostril at a time	Anosmia: URTI, trauma, Parkinson's
CN II — Optic			
☐	Visual acuity	Snellen at 6 m, glasses on, each eye covered; record e.g. 6/6	Pinhole if reduced
☐	Visual fields	1 m apart, cover opposite eyes, red pin from 4 quadrants	Bitemporal / homonymous hemianopia
☐	Fundoscopy*	Ophthalmoscope: disc, vessels, macula	Papilloedema, optic atrophy
CN II + III — Pupils			
☐	Inspect pupils	Size, shape, symmetry	Anisocoria
☐	Direct + consensual	Light from the side into each eye	Absent = afferent/efferent defect
☐	Swinging light	Swing torch between eyes	RAPD (Marcus Gunn)
☐	Accommodation	"Look far away, now at my finger"	Constrict + converge
CN III, IV, VI — Eye movements			
☐	H test	Head still, follow finger slowly in an H; ask about diplopia	III: down + out, ptosis · IV: vertical diplopia · VI: no abduction
☐	Nystagmus	Hold at extremes of gaze	Cerebellar / vestibular
CN V — Trigeminal			
☐	Sensation	Cotton wool (± neurotip) V1 forehead, V2 cheek, V3 jaw; eyes closed	Loss in a division
☐	Motor	Palpate masseter + temporalis on clenching; open jaw against resistance	Jaw deviates to weak side
☐	Corneal reflex*	Cotton wisp to cornea from the side	No blink (V afferent, VII efferent)
☐	Jaw jerk	Finger on chin, mouth slightly open, tap	Brisk = UMN
CN VII — Facial			
☐	Raise eyebrows	"Look up at the ceiling"	Forehead spared = UMN
☐	Eyes shut tight	"Don't let me open them"	Weak closure
☐	Show teeth	"Show me your teeth"	Asymmetric smile
☐	Puff cheeks	"Blow out your cheeks" — press gently	Air escapes on weak side
CN VIII — Vestibulocochlear			
☐	Whisper test	Rub opposite tragus; whisper a number (e.g. 68, 100) at arm's length	Can't repeat = hearing loss
☐	Rinne	512 Hz fork on mastoid, then beside ear: "Which is louder?"	BC > AC = conductive
☐	Weber	Fork on vertex: "Where do you hear it?"	Lateralises (see box)

	Step	What to do / say	Abnormal
CN IX, X — Glossopharyngeal, Vagus			
☐	Palate	"Say Ahh" — palate rises symmetrically; ask about swallow, voice, cough	Uvula deviates AWAY from lesion
☐	Gag reflex*	Verbalise	Absent: IX sensory / X motor
CN XI — Accessory			
☐	Shrug shoulders	Against resistance (trapezius)	Weak on lesion side
☐	Turn head	Against resistance; feel opposite SCM	Weak turn AWAY from lesion side
CN XII — Hypoglossal			
☐	Tongue at rest	Inspect in mouth for wasting, fasciculations	LMN lesion
☐	Protrude + side to side	"Stick out your tongue, move it side to side"	Deviates TOWARDS lesion

4. Closing and presenting

☐	Thank patient, reposition, wash hands
☐	Completion statement: "To complete, I would perform fundoscopy, corneal and gag reflexes, formal smell/taste, a full peripheral neuro exam and audiometry if indicated."
☐	Present: "I examined Mr X, a [age]-year-old… Cranial nerves I–XII were grossly intact. This is a normal cranial nerve examination." If abnormal, lead with the key finding + likely site of lesion.

5. Quick reference

Hearing	Rinne	Weber
Normal	AC > BC both ears	Central
Conductive	BC > AC affected ear	TO affected ear
Sensorineural	AC > BC (both reduced)	AWAY from affected ear

Facial palsy	UMN (stroke)	LMN (Bell's)
Forehead	Spared	Involved
Eye closure	Preserved	Weak, Bell's phenomenon
Lesion side	Contralateral	Ipsilateral

- **Uvula** deviates AWAY from the lesion; **tongue** points TOWARDS it.
- **Ptosis + small pupil** = Horner's · **ptosis + dilated pupil + down-and-out** = CN III (pupil involved: think PComm aneurysm).
- **RAPD:** affected pupil paradoxically dilates when light swings to it = optic nerve lesion.
- **Bulbar (LMN IX–XII):** wasted, fasciculating tongue (MND) · **Pseudobulbar (UMN):** spastic tongue, brisk jaw jerk, emotional lability.
- **CP angle lesion** (acoustic neuroma): V, VII, VIII together; lost corneal reflex is early.
- **Pitfalls:** not asking about pain/diplopia, moving finger too fast, patient moving head, not comparing sides, examining silently.

Upper Limb Neurological Exam — OSCE Checklist

Deakin MD2 · BB6/BB7 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated. Order:* **Inspection → Tone → Power → Reflexes → Coordination → Sensation**. *Always compare* **left vs right** *and narrate aloud.*

1. Introduction (WIPER)

☐	**W**ash hands / hand hygiene
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**E**xpose: top half exposed (singlet / sports bra); maintain dignity, good lighting
☐	Ask about **pain** before touching
☐	Equipment ready: tendon hammer, cotton wool, neurotip (sharps safety, gentle on thin skin), 128 Hz tuning fork

2. Examination

	Step	What to do / say	Root / abnormal
Inspection			
☐	General	Posture, walking aids, scars, skin changes	Hemiplegic posture, deformity
☐	Muscle bulk	Both sides: shoulder girdle, arms, thenar/hypothenar eminences, dorsal interossei	Wasting = LMN; symmetry
☐	Fasciculations	Watch muscles at rest for 10–20 s	LMN (e.g. MND)
☐	Abnormal movements	Tremor (rest/action), myoclonus, chorea, dystonia	Resting tremor = Parkinson's
☐	Pronator drift	"Arms out, palms up, eyes closed" for 20–30 s	Pronates + drifts down = UMN
Tone			
☐	Wrist + elbow	Ask about pain; patient relaxed; passively flex/extend and pronate/supinate at variable speed	Normal / increased / decreased
☐	Rigidity	Feel for cogwheeling (+ tremor) or lead-pipe rigidity	Extrapyramidal (Parkinson's)
☐	Spasticity	Velocity-dependent catch (clasp-knife)	UMN
Power (MRC 0–5) — isolate the joint, match the patient's strength			
☐	Shoulder abduction	"Lift your arms up like wings, don't let me push down"	C5, C6 — deltoid, supraspinatus
☐	Shoulder adduction	"Push your arms down to your sides"	C6, C7, C8 — pec major, lat dorsi
☐	Elbow flexion	"Pull your arm in towards you"	C5, C6 — biceps, brachialis
☐	Elbow extension	"Push me away"	C7, C8 — triceps
☐	Wrist flexion	"Push your wrists down, don't let me push them up"	C6, C7 — FCR, FCU
☐	Wrist extension	"Push your wrists up, don't let me push them down"	C7, C8 — ECR, ECU
☐	Finger extension	"Straighten your fingers, don't let me bend them"	C7, C8 — extensor digitorum
☐	Finger flexion	"Squeeze my two fingers"	C7, C8 — FDP, FDS
☐	Finger abduction	"Spread your fingers, don't let me push them together"	C8, T1 — dorsal interossei
☐	Finger adduction	Resist examiner separating fingers (paper grip)	C8, T1 — palmar interossei
☐	Thumb opposition*	"Touch your thumb to your little finger"	Median nerve (T1)
Reflexes — relaxed, positioned; reinforce (clench teeth) if absent			
☐	Biceps	Arm in lap, forearm pronated; finger on tendon, strike finger	C5, C6
☐	Brachioradialis	Fingers on lateral radius just above wrist; strike	C5, C6
☐	Triceps	Arm loosely supported across chest; strike tendon above olecranon	C7, C8
☐	Finger jerk*	Hand supinated, fingers slightly flexed; strike your own fingers on theirs	C8
☐	Hoffmann's*	Flick distal phalanx of middle finger; watch thumb	Thumb flexes = UMN
Coordination			
☐	Finger–nose	"Touch my finger, then your nose", move target; repeat each side	Past-pointing, intention tremor
☐	Rapid alternating	Pat palm with front then back of the other hand, fast	Dysdiadochokinesia
☐	Rebound	Outstretched arms, eyes closed; tap down — arm returns	Overshoot = cerebellar
Sensation — demonstrate on sternum first, eyes closed, compare sides			
☐	Light touch	Cotton wool: dab each dermatome C5–T1	Dermatomal / glove pattern
☐	Pain	Neurotip: sharp or blunt? Same dermatomes	Spinothalamic loss
☐	Temperature*	Cold tuning fork if indicated	Spinothalamic
☐	Vibration	128 Hz fork on DIP; if absent move to MCP → wrist → elbow	Dorsal column loss
☐	Proprioception	Hold DIP by its sides; "Up or down?"; if absent move proximally	Dorsal column / sensory ataxia

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion statement: "To complete, I would examine the lower limbs and cranial nerves, and check the neck/spine."
☐	Present one finding per domain: "On inspection normal bulk with no fasciculations; tone normal; power 5/5 throughout; reflexes normal and symmetrical; coordination normal; sensation intact. Overall a normal upper limb neurological exam."

4. Quick reference

MRC	Power grade
0	No contraction
1	Flicker, no movement
2	Moves with gravity eliminated
3	Moves against gravity, not resistance
4	Against resistance, weaker than normal
5	Normal power

Dermatome	Test site
C5	Shoulder tip / lateral upper arm
C6	Lateral forearm, thumb
C7	Middle finger
C8	Little finger
T1	Medial upper arm / elbow
Reflexes	0 absent · 1+ reduced · 2+ normal · 3+ brisk · 4+ clonus

Sign	UMN	LMN
Bulk	Normal / disuse	Wasting
Fasciculations	No	Yes
Tone	Increased (spastic)	Decreased
Power	Pyramidal weakness (extensors)	Weak in root/nerve distribution
Reflexes	Brisk, Hoffmann's +	Reduced / absent

Nerve	Classic finding
Radial	Wrist drop; sensory loss first dorsal web space
Ulnar	Claw hand (4th/5th), interossei wasting, Froment's +
Median	Thenar wasting, weak opposition; Tinel's / Phalen's (carpal tunnel)

- **Pyramidal pattern (UMN):** extensors weaker than flexors in the arm — shoulder abduction, elbow and wrist extension go first.
- **Cerebellar signs (DANISH):** Dysdiadochokinesia, Ataxia, Nystagmus, Intention tremor, Slurred speech, Hypotonia — ipsilateral to lesion.
- **Glove distribution** sensory loss = peripheral neuropathy (diabetes, B12, alcohol); **dermatomal** = root; **nerve territory** = mononeuropathy.
- **Pitfalls:** overpowering the patient, not isolating the joint, triceps tested with arm unsupported, skipping reinforcement, testing sensation with eyes open.

Lower Limb Neurological Exam — OSCE Checklist

Deakin MD2 · BB6/BB7 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated. Order:* **Inspection + gait → Tone → Power → Reflexes → Coordination → Sensation**. *Always compare* **left vs right** *and narrate aloud.*

1. Introduction (WIPER)

☐	**W**ash hands / hand hygiene
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**E**xpose: legs exposed (underwear / shorts), cover groin; maintain dignity, good lighting
☐	Ask about **pain** before touching
☐	Equipment ready: tendon hammer, cotton wool, neurotip (sharps safety, gentle on thin skin), 128 Hz tuning fork

2. Examination

	Step	What to do / say	Root / abnormal
Inspection + gait (patient standing first)			
☐	General	Walking aids, orthoses, footwear wear, scars (back, legs), skin	Deformity, pes cavus
☐	Muscle bulk	Both sides: quadriceps, calves, tibialis anterior, small foot muscles	Wasting = LMN; symmetry
☐	Fasciculations	Watch thighs and calves at rest	LMN (e.g. MND)
☐	Gait	Walk to wall and back: arm swing, stride, base, turning	Hemiplegic, shuffling, ataxic, high-stepping
☐	Heel walk	"Walk on your heels"	Can't = L4/L5, foot drop
☐	Toe walk	"Walk on your tiptoes"	Can't = S1
☐	Tandem walk	"Heel to toe in a straight line"	Ataxia (cerebellar)
☐	Romberg's	Feet together, arms by side, eyes open then closed; stand close to catch	Sway with eyes closed only = sensory ataxia
Tone (patient lying on bed)			
☐	Leg roll	Roll the leg side to side; foot should flop	Stiff = increased tone
☐	Knee lift	Quickly lift behind knee	Heel lifts off bed = increased tone
☐	Knee + ankle	Passively flex/extend at variable speed	Spasticity / rigidity / hypotonia
☐	Ankle clonus	Knee slightly bent; sharply dorsiflex and hold	≥ 3–5 beats = UMN
Power (MRC 0–5) — isolate the joint, match the patient's strength			
☐	Hip flexion	"Keep your leg straight, lift it up, don't let me push it down"	L2, L3 — iliopsoas
☐	Hip extension	"Push my hand down into the bed" (hand behind calf)	L5, S1, S2 — gluteus maximus
☐	Hip abduction	"Push your legs out against my hands"	L4, L5, S1 — gluteus medius/minimus
☐	Hip adduction	"Squeeze your legs together"	L2, L3, L4 — adductors
☐	Knee flexion	Knee bent: "Pull your heel towards your bottom"	L5, S1 — hamstrings
☐	Knee extension	Knee bent: "Push me away"	L3, L4 — quadriceps
☐	Ankle dorsiflexion	"Pull your foot up, don't let me push it down"	L4, L5 — tibialis anterior
☐	Ankle plantarflexion	"Push down against my hand" (like a pedal)	S1, S2 — gastrocnemius, soleus
☐	Inversion	Hold foot turned in against resistance	L5, S1 — tibialis posterior
☐	Eversion	Hold foot turned out against resistance	L5, S1 — peronei
☐	Great toe extension	"Push your toe up against my finger"	L5, S1 — EHL
☐	Great toe flexion*	"Push your toe down against my finger"	S1, S2 — FHL / FDL
Reflexes — relaxed, positioned; reinforce (clench teeth, Jendrassik) if absent			
☐	Knee jerk	Support both knees slightly flexed; strike patellar tendon	L3, L4
☐	Ankle jerk	Hip externally rotated, knee bent, dorsiflex foot slightly; strike Achilles	S1, S2
☐	Plantar (Babinski)	Blunt object up lateral sole then across; report **first** movement of big toe	Upgoing = UMN
Coordination			
☐	Heel–shin	"Heel on opposite knee, slide down the shin, lift, repeat"	Ataxia, intention tremor
☐	Toe–finger*	"Touch my finger with your big toe"	Past-pointing
☐	Foot tapping	"Tap your foot quickly on my hand"	Dysdiadochokinesia
Sensation — demonstrate on sternum first, eyes closed, compare sides			
☐	Light touch	Cotton wool: dab each dermatome L2–S1	Dermatomal / stocking pattern
☐	Pain	Neurotip: sharp or blunt? Same dermatomes	Spinothalamic loss
☐	Temperature*	Cold tuning fork if indicated	Spinothalamic

	Step	What to do / say	Root / abnormal
☐	Vibration	128 Hz fork on great toe tip; if absent move to MTP → medial malleolus → tibial tuberosity	Dorsal column loss
☐	Proprioception	Hold great toe by its sides; "Up or down?"; if absent move proximally	Dorsal column / sensory ataxia

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion statement: "To complete, I would examine the upper limbs and cranial nerves, examine the spine, and check saddle sensation and anal tone if indicated."
☐	Present one finding per domain: "Gait normal and Romberg's negative; normal bulk; tone normal with no clonus; power 5/5 throughout; reflexes normal, plantars downgoing; coordination normal; sensation intact. Overall a normal lower limb neurological exam."

4. Quick reference

MRC	Power grade
0	No contraction
1	Flicker, no movement
2	Moves with gravity eliminated
3	Moves against gravity, not resistance
4	Against resistance, weaker than normal
5	Normal power

Dermatome	Test site
L2	Anterior upper thigh
L3	Medial knee
L4	Medial leg / medial malleolus
L5	Dorsum of foot, great toe
S1	Lateral foot, little toe, heel
Reflexes	0 absent · 1+ reduced · 2+ normal · 3+ brisk · 4+ clonus

Sign	UMN	LMN
Bulk	Normal / disuse	Wasting
Fasciculations	No	Yes
Tone	Increased, clonus	Decreased
Power	Pyramidal weakness (flexors)	Weak in root/nerve distribution
Reflexes	Brisk, plantar upgoing	Reduced / absent, plantar down

Gait	Think of
Hemiplegic	Circumduction — stroke
Shuffling	Small steps, reduced arm swing — Parkinson's
Broad-based ataxic	Cerebellar lesion
High-stepping	Foot drop — common peroneal / L5
Waddling	Proximal myopathy

- **Pyramidal pattern (UMN):** flexors weaker than extensors in the leg — hip flexion, knee flexion and dorsiflexion go first.
- **Foot drop:** common peroneal = weak dorsiflexion + eversion, **inversion spared**; L5 root = inversion also weak.
- **Romberg's positive** = proprioceptive (dorsal column) loss, not cerebellar. **Stocking** loss = peripheral neuropathy.
- **Red flags:** saddle anaesthesia, urinary retention, bilateral leg weakness = cauda equina — urgent MRI.
- **Pitfalls:** not offering to catch in Romberg's, overpowering the patient, skipping reinforcement, testing sensation with eyes open.

Eye History — OSCE Checklist

Deakin MD2 · BB8 · Ophthalmology

Tick each item as you ask it. **Shaded rows (*)** *are asked if relevant.* † *= added from standard teaching. Use a person-centred approach (Calgary-Cambridge). Ask* **unilateral vs bilateral** *for every symptom.*

1. Opening

☐	Introduce self, confirm patient ID, consent
☐	Open question: "What's brought you in about your eyes today?"

2. Presenting complaint

	Ask about	Example	Significance
☐	Laterality	One eye or both?	
☐	Onset and duration	Sudden or gradual? How long?	Sudden = urgent
☐	Previous episodes	Has this happened before?	Recurrent uveitis, HSV
☐	Vision change	Reduced acuity, blurring, diplopia, field defect	Acute loss = red flag
☐	Flashes and floaters	New flashes, floaters, "curtain" over vision	Retinal tear / detachment
☐	Redness	Where? Diffuse or around the cornea?	Unilateral red eye = red flag
☐	Pain	Severity, constant vs intermittent	Constant pain = red flag
☐	Discharge	Watery, purulent, crusting	Viral vs bacterial conjunctivitis
☐	Photophobia	Light sensitivity	Keratitis, uveitis, meningitis
☐	Sensation	Gritty, foreign-body feeling, itch, dryness	FB, abrasion, allergy, dry eye

3. Eye-specific background

☐	Vision aids	Glasses, contact lenses (overnight wear?)	Contact lens keratitis
☐	Previous eye surgery	Laser, cataract, intravitreal injections, retinal detachment repair	Recent procedure = red flag
☐	Previous trauma	Any past eye injury	
☐	Last review	Last optometry / ophthalmology review	

4. Trauma history*

☐	High-speed injury	Drilling, hammering, grinding, explosion	Beware painful impact then painless eye — penetrating FB
☐	Sharp object	Glass, nail, metal	Globe rupture
☐	Contaminant	Plant material	Fungal keratitis
☐	Chemical	Acid or alkali	Alkali = worst; beware the "white eye"
☐	PPE	Eye protection worn?	

5. Systemic review

	Ask about	Think of
☐	Fever, nausea, vomiting, headache	Acute angle closure, orbital cellulitis
☐	Jaw claudication, scalp tenderness, age >50	Giant cell arteritis
☐	Weight loss, night sweats, rashes, recent viral illness	Systemic or infective cause
☐	Vesicular rash / girdle pain on face	HZO — check Hutchinson's sign (nose tip)
☐	Neurological: speech, vertigo, cognition, balance, weakness, sensory change, diplopia	Stroke, space-occupying lesion

6. Past history, medications, family and social

☐	PMH	Diabetes, CVD, hypertension, autoimmune / connective tissue disease, immunosuppression, HSV/VZV
☐	Medications	COCP, corticosteroids, biologics, α- and β-blockers, anticholinergics, eye drops
☐	Allergies	Drug allergies †
☐	Family history	Glaucoma (incl. angle closure), macular degeneration, strabismus / amblyopia
☐	Social	Smoking, occupation and PPE, **driving**
☐	Risk factors	Age, myopia / hypermetropia, prematurity, previous eye trauma

7. Close

☐	Ideas, concerns, expectations (e.g. impact on driving, work)
☐	Summarise, check accuracy, invite questions, thank patient

Eye Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General + vitals → Visual acuity → Visual fields → Pupils → Eye movements → Fundoscopy → Palpation**. *Order may change with the presentation. Always test* **each eye separately**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Position: seated, at eye level, ~1 m apart; glasses available; dim room for pupils/fundoscopy
☐	Equipment: Snellen chart, pinhole, red-headed pin, pen torch, ophthalmoscope ± Ishihara plates

2. Examination

	Step	What to do / say	Abnormal
General inspection and vital signs			
☐	Vital signs	BP, HR, temp etc.	Required in every eye documentation
☐	Ptosis	Compare lid heights	CN III, Horner's, myasthenia
☐	Exophthalmos	Look from front, side, above	Thyroid eye disease, orbital mass
☐	Lids	Swelling, lesions, stye, chalazion, blepharitis	Periorbital / orbital cellulitis
☐	Conjunctiva and sclera	Yellow (jaundice), blue (osteogenesis imperfecta), pallor (anaemia), redness — distribution	Ciliary flush = keratitis, uveitis, glaucoma †
☐	Slit lamp*	Corneal ulceration, anterior chamber (demonstrated in session)	
Visual acuity			
☐	Snellen	6 m, glasses on, cover one eye, read smallest line; repeat other eye	Record e.g. 6/9 (distance / line) †
☐	Pinhole	If acuity reduced	Improves = refractive error
☐	Poor acuity*	Count fingers → hand movements → perception of light †	
Visual fields (revision from CN exam)			
☐	Confrontation	1 m apart, cover opposite eyes; red pin in from 4 quadrants: "Tell me when you first see red"	Hemianopia, quadrantanopia
☐	Blind spot*	Move pin across centre of field	Enlarged = papilloedema; central scotoma
☐	Red desaturation*	Compare red colour with each eye	Washed-out = optic nerve pathology
☐	Finger wiggle*	If acuity too poor for the pin	Quick screen
Pupils (dim room, patient looks into distance)			
☐	Inspect	Size (normal 2–6 mm), shape, equality	Anisocoria, irregular pupil
☐	Direct and consensual	Light into each eye in turn; watch both pupils	Afferent vs efferent defect
☐	Swinging light	Swing torch between eyes	RAPD (paradoxical dilation) = optic nerve
☐	Near response	Look far, then at a near target	Constriction + convergence
Eye movements (CN III, IV, VI)			
☐	Diplopia	Ask before and during	Binocular diplopia = red flag
☐	H test	Head still; look left, right, up, down; conjugate?	Palsy pattern
☐	Nystagmus	At extremes of gaze	Vestibular, cerebellar
☐	Cover test*	If diplopia: does it go with either eye covered?	Persists with one eye covered = monocular — ask for help
Other tests			
☐	Colour vision*	Ishihara plates (demonstrated in session)	Optic neuritis, congenital deficiency
☐	Corneal reflex*	Rarely routine (ICU / brain death, major trauma)	CN V / VII
☐	Fundoscopy	Red reflex; disc (colour, margins, cup), vessels, macula — dilated pupils in session	Absent red reflex = red flag; papilloedema, retinopathy
Palpation			
☐	Orbits	Gently palpate orbital rims	Tenderness, step (fracture)
☐	Pre-auricular nodes	Palpate in front of tragus	Viral / chlamydial conjunctivitis

3. Closing and documenting

☐	Thank patient; wash hands
☐	**Everyone**: document vital signs, general inspection, visual acuity, pupils and fundoscopy
☐	**Visual loss / change or neuro signs**: also visual fields, eye movements and cranial nerve examination
☐	Present: "Visual acuity 6/6 both eyes; fields full to confrontation; pupils equal and reactive with no RAPD (PERRLA); full eye movements without diplopia or nystagmus; red reflex present with normal fundi."
☐	Any abnormal eye exam → seek help / refer (optometrist, ophthalmologist, ED)

4. Quick reference

Area	Red flags — refer
Eye	Unilateral red eye, constant pain, acute vision loss, binocular diplopia, abnormal pupil / RAPD, absent red reflex, abnormal cornea, restricted movements
History	Trauma / FB / high-risk activity, recent eye surgery, only seeing eye, previous retinal detachment, neonatal conjunctivitis, non-resolving lid lesion
Associated	Nausea, photophobia, headache, jaw claudication, facial HSV/VZV rash, Hutchinson's sign, painless transient visual loss

Until proven otherwise	
Flashes + floaters	Retinal tear
Vision change / diplopia, age >50	Giant cell arteritis
Serious periocular trauma	Ruptured / penetrated globe; intracranial injury
Periocular laceration	Contains a foreign body
Eyelid laceration	Full thickness, involves globe

• **PERRLA** = Pupils Equal, Round, React to Light, Accommodate.

• Pupil pathway: afferent CN II → pretectal nucleus → both Edinger–Westphal nuclei → efferent CN III (parasympathetic, miosis). Sympathetic (dilation): upper thoracic → superior cervical ganglion → along internal carotid.

Ear Examination — OSCE Checklist

Deakin MD2 · BB4 · ENT

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Inspect → Palpate → Otoscopy → Whisper test → Weber → Rinne**. *Examine the* **good ear first**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask which ear is affected; position seated, ear at your eye level
☐	Equipment: otoscope with range of speculae, 512 Hz tuning fork

2. Examination

	Step	What to do / say	Abnormal
Inspection (external ear)			
☐	Pinna	Size, shape, deformity; trauma (haematoma)	Cauliflower ear
☐	Pre-auricular region	Cartilage tags, sinuses or pits	Syndromes with hearing loss
☐	Skin	Suspicious lesions (SCC, BCC), skin conditions of pinna and canal	Neoplasia, eczema
☐	Scars	Pre- or post-auricular	Previous surgery
☐	Hearing aids	Note and remove before otoscopy	
☐	Discharge	Mucopurulent, serous, blood-stained	Otitis externa / media, perforation
Palpation			
☐	Tragus and pinna	Gently move — tenderness?	Tender = otitis externa
☐	Mastoid	Palpate behind the ear	Tender = mastoiditis (with otitis media)
Otoscopy			
☐	Speculum	Largest that fits comfortably	
☐	Grip	Hold otoscope like a pen; hand braced against patient's head	Prevents injury if head moves
☐	Straighten canal	Adult: pull pinna up and back; child: pull back only	
☐	Canal	Inflamed? Wax? Discharge? Foreign body?	Otitis externa
☐	Tympanic membrane	Colour, light reflex, short process and handle of malleus, umbo, pars flaccida; move to see whole TM	Bulging, retraction, perforation, grommet
Hearing			
☐	Whisper test	Mask other ear (rub tragus / fingers); whisper numbers; increase volume if needed; each side	Formal audiology if abnormal
☐	Weber (512 Hz)	Strike fork; base on vertex or midline forehead: "Louder on one side or the middle?"	Lateralises (see table)
☐	Rinne (512 Hz)	Base on mastoid until no longer heard, then prongs beside the ear: "Can you hear it now?"	Positive (AC > BC) = normal

3. Closing

☐	Thank patient; clean otoscope speculum; wash hands
☐	Present: "Pinnae normal, no tenderness; canals clear; both TMs intact and pearly grey with a normal light reflex; whisper test normal; Weber central, Rinne positive bilaterally."
☐	Completion: "I would complete the ENT exam (nose, throat, neck nodes) and arrange formal audiometry if indicated."

4. Quick reference

Weber / Rinne (Deakin table)	Weber central	Weber lateralises LEFT	Weber lateralises RIGHT
Rinne AC > BC both ears	Normal	Sensorineural loss RIGHT	Sensorineural loss LEFT
Rinne BC > AC left	—	Conductive loss LEFT	Combined loss
Rinne BC > AC right	—	Combined loss	Conductive loss RIGHT

TM appearance †	Think of
Pearly grey, light reflex	Normal
Red, bulging, lost landmarks	Acute otitis media
Dull, retracted ± fluid level / bubbles	Otitis media with effusion
Hole in TM	Perforation
Small tube in TM	Grommet (not a foreign body)
Swollen canal, debris, tender tragus	Otitis externa

- **Conductive** loss: Weber goes TO the bad ear, Rinne negative (BC > AC).
- **Sensorineural** loss: Weber goes AWAY from the bad ear, Rinne positive (both reduced).
- **False-negative Rinne**: a dead ear hears BC louder via the other cochlea — Weber clarifies.
- Rinne negative needs a conductive loss of ~20 dB or more.

Nose & Sinus Examination — OSCE Checklist

Deakin MD2 · BB4 · ENT

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Inspect externally → Palpate → Airflow → Anterior rhinoscopy → Sinuses**. *Nasal speculum is rarely used outside specialist settings — a pen torch or otoscope is usual.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask about nasal symptoms (blockage, discharge, bleeding, smell); position seated at eye level
☐	Equipment: pen torch, cold metal tongue depressor, nasal speculum or otoscope with wide speculum

2. Examination

	Step	What to do / say	Abnormal
External inspection			
☐	Front and side	Swelling, deformity, scars or creases, redness / skin disease	Trauma, rhinophyma, BCC
☐	From above	Patient tilts head back; look down the bridge	Deviated nose
☐	Discharge	Rhinorrhoea, crusting; unilateral?	Unilateral offensive in a child = foreign body
Palpation			
☐	Nasal bones	Gently palpate bridge	Tenderness, crepitus, step = fracture
Airway patency			
☐	Misting test	Cold metal tongue depressor under nose as patient breathes out	Condensation under both nostrils = airflow
☐	Sniff test	Occlude one nostril, ask patient to sniff; repeat	Unilateral obstruction
Anterior rhinoscopy			
☐	Without speculum	Tip nose up with thumb; look into the anterior nares with torch	
☐	With speculum / otoscope	Gently open; look straight back, not up	
☐	Septum	Straight or deviated? Bleeding points, clots, crusting, perforation; septal haematoma	Little's area = common epistaxis site †; haematoma = urgent drainage
☐	Turbinates	Inflammation, swelling, colour	Rhinitis (pale, boggy = allergic †)
☐	Polyps*	Grey, glistening, insensate †	Chronic rhinosinusitis
Sinuses			
☐	Palpate	Frontal (above brow) and maxillary (cheek) sinuses	Tenderness = sinusitis
☐	Transilluminate*	Pen torch in a dark room (demonstrated in session)	Reduced = fluid / mass
☐	Note	Ethmoid and sphenoid sinuses cannot be examined physically	

3. Closing

☐	Thank patient; clean equipment; wash hands
☐	Present: "No external deformity; both nasal airways patent; septum midline with no haematoma or perforation; turbinates normal; no sinus tenderness."
☐	Completion: "I would complete the ENT exam, test smell (CN I) if indicated, and consider nasendoscopy."

Throat & Mouth Examination — OSCE Checklist

Deakin MD2 · BB4 · ENT

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Inspect mouth → Tongue → Tonsils/pharynx → Palpate neck**. *Use a bright pen torch with the mouth wide open.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain the exam and gain consent; ask about pain
☐	Ask about sore throat, swallowing, voice; remove dentures if present †
☐	Equipment: pen torch, tongue depressor, gloves

2. Examination

	Step	What to do / say	Abnormal
Oral cavity			
☐	Lips and mucosa	Ulcers, lesions, colour	Aphthous ulcers, candida, malignancy †
☐	Dentition	Comment on teeth and gums	Dental caries, abscess
☐	Tongue	Appearance; ask to raise tongue to check underneath if indicated	Lesions, floor-of-mouth swelling
Oropharynx			
☐	"Say ahh"	Mouth wide, tongue protruded	Palate rises symmetrically; uvula central †
☐	Tongue depressor	If tonsils not seen: press middle third of tongue, not the back †	
☐	Tonsils	Size, symmetry, inflammation, exudate	Tonsillitis; asymmetry = quinsy or malignancy †
☐	Pharynx	Swelling, inflammation, exudate, post-nasal drip	
Neck (if clinically indicated)			
☐	Lymph nodes	From behind: submental, submandibular, tonsillar, cervical chain, supraclavicular, pre/post-auricular, occipital	Infection, malignancy
☐	Parotids*	Palpate in front of ear and over angle of jaw	Parotitis, tumour

3. Closing

☐	Thank patient; dispose of tongue depressor; wash hands
☐	Present: "Good dentition; tongue and oral mucosa normal; tonsils symmetrical without erythema or exudate; uvula central; no cervical lymphadenopathy."
☐	Completion: "I would complete the ENT exam and consider a throat swab if indicated."

4. Quick reference †

- **Peritonsillar abscess (quinsy)**: unilateral tonsil swelling, uvula deviated away, trismus, "hot potato" voice, drooling — urgent ENT referral.
- **Airway red flags**: stridor, drooling, unable to swallow saliva, tripod posture — do not examine with a tongue depressor; call for senior help.
- Uvula deviates **away** from a CN X lesion (links to cranial nerve exam).

Respiratory Examination — OSCE Checklist

Deakin MD2 · CR7 · Time: 10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Environment → General → Vitals → Hands → Arms → Face → Trachea → Nodes → Posterior chest → Anterior chest**. *Deakin order: examine the* **back first**, *then the front. Always compare side to side.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain and gain consent: upper body clothing removed, only exposed when needed
☐	Offer a chaperone; privacy; warmth
☐	Position: sitting at 45° † (sit forward for the back)
☐	Equipment: stethoscope

2. Examination

	Step	What to do / say	Abnormal
Environment and general inspection			
☐	Environment	Oxygen (device, flow), inhalers, spacers, sputum pot, nebuliser †	Chronic lung disease
☐	Respiratory effort	Rate, accessory muscle use, tripod position, pursed lips †	Respiratory distress
☐	Speech	How much can they say in one breath?	Full sentences vs words
☐	Audible sounds	Cough, wheeze, stridor, voice change	Obstruction, recurrent laryngeal nerve
Vital signs			
☐	Full set	RR (count 30 s+), SpO_2 (on air?), HR, BP, temp	
Hands			
☐	Nails	Clubbing (look side-on, Schamroth's window †)	Lung cancer, bronchiectasis, ILD, CF
☐	Nicotine staining	Fingers and nails	Smoking
☐	Palms	Pallor	Anaemia
☐	Tremor	Fine tremor; flap if CO_2 retention †	Salbutamol use; hypercapnia
Arms			
☐	Skin	Bruising, thin skin	Long-term corticosteroids
Face			
☐	Eyes	Conjunctival pallor; Horner's syndrome (ptosis, miosis)	Anaemia; Pancoast tumour
☐	Sinuses	Facial tenderness	Sinusitis
☐	Mouth	Central cyanosis (under tongue)	Hypoxaemia
Neck			
☐	Trachea	Midline position (warn patient); tracheal tug	Deviation: pneumothorax, collapse, effusion
☐	Lymph nodes (from behind)	8 groups: submental, submandibular, anterior cervical, posterior cervical, pre-auricular, post-auricular, occipital, supraclavicular	Malignancy, infection
☐	JVP*	Raised JVP †	Cor pulmonale
Posterior chest (patient sitting forward, arms crossed †)			
☐	Inspect	Shape, symmetry, scars (thoracotomy, drains), muscle wasting, kyphoscoliosis	Previous surgery, restrictive disease
☐	Expansion	Hands on lower ribs, thumbs meeting in midline; deep breath — watch thumbs separate †	Reduced side = pathology
☐	Rib tenderness	Palpate ribs laterally	Fracture, costochondritis
☐	Percussion	Compare side to side, top to bottom, including axillae †	Dull: consolidation, collapse; stony dull: effusion; hyper-resonant: pneumothorax
☐	Breath sounds	Diaphragm, side to side, top to bottom; patient breathes through mouth	Vesicular vs bronchial; reduced; added sounds
☐	Added sounds	Crackles (fine/coarse), wheeze, pleural rub	Fibrosis, oedema, infection; airway narrowing
☐	Vocal resonance	"Say 99" while auscultating each area †	Increased = consolidation; reduced = effusion
Anterior chest (patient back at 45°)			
☐	Inspect	Look again for increased work of breathing, chest shape (barrel, pectus), scars	Hyperinflation (COPD)
☐	Palpate	Subcutaneous emphysema (crackling under fingers); apex beat †	Air leak; mediastinal shift
☐	Percussion	Include supraclavicular region and clavicles	As above
☐	Auscultation	Breath sounds, added sounds, vocal resonance including apices	As above
Legs*			
☐	Oedema, calves	Pitting oedema; calf swelling/tenderness †	Cor pulmonale; DVT (PE risk)

3. Closing and presenting

☐	Help patient dress; leave them as they arrived; wash hands
☐	If asked a question you can't answer, say you'll pass it to someone senior
☐	Completion: "I would review the observation chart, do a peak flow / spirometry, examine sputum, and request a chest X-ray †."
☐	Present: "Mrs X is comfortable at rest on room air, RR 14, SpO_2 98%. No clubbing or peripheral signs of respiratory disease. Trachea midline, expansion equal, percussion resonant throughout, vesicular breath sounds with no added sounds."

4. Quick reference †

Condition	Expansion	Trachea	Percussion	Breath sounds	Vocal resonance
Consolidation (pneumonia)	↓ affected side	Midline	Dull	Bronchial ± crackles	↑
Pleural effusion	↓ affected side	Away (if large)	Stony dull	↓ or absent	↓
Pneumothorax	↓ affected side	Away (tension)	Hyper-resonant	↓ or absent	↓
Collapse (lobar)	↓ affected side	Towards	Dull	↓	↓
COPD	↓ both, hyperinflated	Midline (tug)	Hyper-resonant	↓ ± wheeze	Normal / ↓
Pulmonary fibrosis	↓ both	Midline	Normal	Fine end-inspiratory crackles	Normal / ↑

- **Stridor** = upper airway (inspiratory); **wheeze** = lower airway (expiratory). Stridor is an emergency.
- Clubbing is **not** a feature of uncomplicated COPD — look for another cause (e.g. lung cancer).

Abdominal (GI) Examination — OSCE Checklist

Deakin MD2 · RG5 · Time: 10–12 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Vitals → Hands → Arms → Face → Chest → Abdomen (inspect, palpate, percuss, auscultate) → Hernias → Complete**. *Watch the patient's face throughout and examine any painful area* **last**.

1. Introduction

☐	Wash hands; introduce self and the exam; confirm patient ID
☐	Explain and gain consent; ask about pain (examine that area last)
☐	Offer a chaperone; privacy (curtains); quiet, well-lit, warm room
☐	Position: lying flat on **one pillow**, arms by sides
☐	Expose abdomen; cover upper and lower body with 2 blankets (ideally nipples to mid-thigh †)
☐	Equipment: stethoscope, tape measure, torch

2. Examination

	Step	What to do / say	Abnormal
General inspection			
☐	Jaundice	Skin and sclera	Liver disease, haemolysis, obstruction
☐	Skin	Discoloration, pigmentation, rashes, acanthosis nigricans	
☐	Pain and position	Lying very still / writhing / sitting forward	Peritonitis / renal colic / pancreatitis
☐	Body habitus	Wasting or obesity	Malignancy, malabsorption
☐	Mental state	Confusion	Hepatic encephalopathy
Vital signs			
☐	Pulse (manual)	Rate, rhythm, character	Tachycardia: sepsis, bleeding
☐	BP lying and standing	Postural drop	Hypovolaemia (GI bleed)
☐	RR, temp, SpO_2, BGL	Full set; height and weight if possible	
Hands (both)			
☐	Nails	Leukonychia, clubbing	Hypoalbuminaemia; IBD, cirrhosis
☐	Palms	Pale palmar creases; palmar erythema	Anaemia; liver disease (may be normal)
☐	Dupuytren's contracture	Thickened palmar fascia, fixed finger flexion	Alcohol-related liver disease
☐	Asterixis	Arms out, wrists cocked back, fingers spread, ≥20 seconds	Liver failure ("flap")
Arms			
☐	Skin	Bruising, petechiae, scratch marks, spider naevi	Coagulopathy, cholestasis (pruritus), chronic liver disease
☐	Muscle	Wasting	Malnutrition
Face			
☐	Eyes	Pale conjunctiva, scleral jaundice, xanthelasma	Anaemia; liver disease; cholestasis
☐	Salivary glands	Parotid and submandibular enlargement	Alcohol, sialadenitis
☐	Mouth (torch)	Angular stomatitis, dentition, oral candidiasis	Iron/B deficiency, immunosuppression
☐	Lymph nodes*	Left supraclavicular (Virchow's) node †	Gastric malignancy (Troisier's sign)
Chest			
☐	Spider naevi	>5 in SVC distribution †	Chronic liver disease
☐	Gynaecomastia*	Verbalise — not expected to examine	Chronic liver disease, spironolactone
Abdomen — inspection (stand back, then from the side)			
☐	Scars, stoma	Surgical or trauma scars; laparoscopic ports	Previous surgery
☐	Distension	5 Fs: fat, fluid, faeces, flatus, fetus	Ascites, obstruction
☐	Localised swelling	Mass, bowel loop, enlarged organ	
☐	Veins and pulsation	Prominent veins; visible pulsation or peristalsis	Portal hypertension; AAA; obstruction
☐	Cough*	Ask patient to cough — watch hernial orifices and for pain †	Hernia; peritonism
Abdomen — palpation (warm hands, kneel to bed level †, watch face)			
☐	Light palpation	All 9 regions, flat of hand, painful area last	Tenderness, guarding, rigidity, rebound
☐	Deep palpation	All 9 regions again	Masses — describe (see box)
☐	Liver	Start RIF, hand parallel to right costal margin; patient breathes in through mouth; move up 1–2 cm on each breath out	Edge: hard/soft, tender, regular, pulsatile
☐	Spleen	Left hand behind left lower ribs; right hand from RIF towards left costal margin	Any palpable spleen is abnormal
☐	Kidneys	Bimanual ballot each side	Normal kidneys are impalpable
☐	Aorta	Both hands either side above umbilicus	Expansile + pulsatile = AAA

	Step	What to do / say	Abnormal
☐	Bladder*	Palpate suprapubic area if retention suspected	Dull to percussion
Abdomen — percussion			
☐	Liver span	Right MCL from above 6th rib (resonant → dull) down to palpable edge	Normal <12 cm
☐	Spleen*	Percuss left costal margin / Traube's space †	
☐	Shifting dullness	Percuss from midline to flank until dull; mark; roll patient towards you, wait 30 s, re-percuss †	Dullness shifts = ascites
☐	Bladder*	If urinary retention suspected	Dullness above pubis
Abdomen — auscultation			
☐	Bowel sounds	Just below umbilicus	Absent (ileus, peritonitis) / tinkling (obstruction †)
☐	Renal bruits	Either side of midline at level of umbilicus (check BP if present)	Renal artery stenosis
☐	Aortic bruit*	Above umbilicus †	Atherosclerosis, AAA
Hernias and groin			
☐	Hernial orifices	Strain or cough: midpoint of inguinal ligament and lateral to pubic tubercle; examine standing	Inguinal / femoral hernia
☐	Inguinal lymph nodes	Both sides	Lymphadenopathy
Legs*			
☐	Oedema	Ankle and sacral pitting oedema †	Hypoalbuminaemia, liver disease

3. Closing and presenting

☐	Completion: "I would also examine the external genitalia, perform a DRE, and dipstick the urine."
☐	Ask about concerns/questions; check understanding
☐	Cover patient, help them dress, leave them comfortable
☐	Put equipment away; wash hands
☐	Present: "Mr X is comfortable at rest with no peripheral stigmata of chronic liver disease. The abdomen is soft and non-tender with no organomegaly or masses, no shifting dullness, and normal bowel sounds."

4. Quick reference

Mass — describe	
Site	Region
Size, shape	cm
Surface, edge	Regular / irregular
Consistency	Hard / soft
Tenderness	
Mobility	Mobile / fixed; moves with breathing?
Pulsatile?	Expansile = aneurysm
Get above it?	Can't get above liver or spleen
Percussion	Resonant / dull

Scar	Operation
Upper midline	Laparotomy (upper GI)
Right subcostal (Kocher's)	Open cholecystectomy
Right paramedian	Older laparotomy
Lower midline	Laparotomy (lower GI, gynae)
Loin	Nephrectomy
RIF gridiron	Appendicectomy
Iliac fossa J-shape	Kidney transplant
Pfannenstiel	Caesarean, gynae
Umbilical port	Laparoscopic surgery

Spleen vs left kidney †	Spleen	Kidney
Notch	Present	Absent
Get above	No	Yes
Moves with breath	Down and medially	Little
Ballotable	No	Yes
Percussion	Dull	Resonant (bowel in front)

- **Hepatomegaly causes †**: congestion (heart failure), infiltration (fatty liver, malignancy), infection (hepatitis).
- **Splenomegaly causes †**: portal hypertension, haematological malignancy, infection (EBV, malaria).
- **Chronic liver disease signs**: clubbing, leukonychia, palmar erythema, Dupuytren's, spider naevi, gynaecomastia, jaundice, ascites, caput medusae.
- Pitfalls: more than one pillow, not watching the face, palpating tender area first, forgetting hernias.

Digital Rectal Examination (DRE) — OSCE Checklist

Deakin MD2 · RG5 · Part-task trainer

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Consent → Position → Inspect → Strain → Insert → Sweep walls → Anterior wall → Tone → Inspect glove → Close**. *Treat the model as a real patient: communicate, reassure and explain what they will feel.*

1. Before you start

☐	Wash hands; fingernails short; introduce self; confirm patient ID
☐	Informed consent **before** patient undresses: why, what you will do, position, what they will feel
☐	Do not assume willingness because they have not objected; allow time for questions
☐	Offer a chaperone; privacy (curtains); quiet, well-lit, warm room
☐	Check cautions — seek senior advice if: anal fissure, thrombosed prolapsed haemorrhoids, child, active IBD / diverticulitis, recent surgery/trauma/radiotherapy, immunocompromised or bleeding risk, history of abuse
☐	Equipment: **2 pairs** of non-sterile gloves, lubricant, light, bluey, tissues
☐	Position: **left lateral**, knees drawn up to chest; cover with gown/sheet until exposure needed

2. Procedure

	Step	What to do / say	Abnormal
Inspect (gloved, separate buttocks)			
☐	Perianal area	Skin tags, erythema, excoriation, warts (condylomata), fissure, external/thrombosed haemorrhoids, prolapse, carcinoma	
☐	Strain	"Push down as if opening your bowels"	Incontinence, perineal descent, haemorrhoids, prolapse
Insert			
☐	Warn the patient	"You may feel like you need to open your bowels — it's just the feeling of my finger"	
☐	Lubricate	Lubricate right index finger; lift right buttock with left hand	
☐	Position finger	Pad of finger on anus, pointing towards patient's front	
☐	Relax and enter	Patient breathes deeply through mouth; wait for sphincter to relax, then gently insert, turning finger towards head	Internal sphincter tone assessed on entry
☐	Resistance	Stop, hold still, let patient breathe slowly; do **not** force. Stop if spasm and severe pain	Fissure, stricture, fibrosis, carcinoma
Palpate			
☐	Rectal walls	Sweep pulp from 2 o'clock clockwise to 10 o'clock at 3 levels (~7–8 cm); feel sacral hollow and coccyx	Mass, polyp, ulcer with hard raised edge
☐	Anterior wall — male	10 to 2 o'clock: prostate size, consistency, median sulcus, nodules, tenderness ("you may feel you need to pass urine")	See box
☐	Anterior wall — female	Cervix felt as a smooth lump; retroverted uterus may press on wall	Pelvic mass
☐	Strain again	Patient pushes down; sweep to feel an extra centimetre	
☐	Contents	Consistency of faeces	Hard impacted stool, foreign body
☐	Squeeze	"Relax… now squeeze my finger… now relax"	External sphincter tone; weak = sphincter injury or neurological
After removal			
☐	Inspect glove	Colour, blood, mucus, pus	Fresh blood = haemorrhoids, rectal lesion; black = upper GI bleed, iron
☐	FOBT*	Faecal occult blood test if indicated	

3. Closing

☐	Offer tissues / help wiping; remove outer gloves when soiled
☐	Dispose of waste in contaminated bin
☐	Cover patient; tell them it's finished; offer help dressing
☐	Simple explanation of findings; if unsure, say you'll confirm with the treating doctor
☐	Invite questions; check understanding; wash hands
☐	Document consent, chaperone and all findings

4. Quick reference

Finding	Think of
Loss of tone, patulous anus	Sphincter injury, cord compression / cauda equina
Hard impacted stool	Constipation, overflow incontinence
Irregular mucosa / mass	Rectal cancer
Smooth, large, firm, non-tender prostate	BPH
Hard irregular nodule / fixed mass	Prostate cancer, chronic prostatitis
Large, boggy, tender prostate	Acute prostatitis
Mass on anterior wall (female)	Ovarian or uterine mass

#	Indications
1	Rectal bleeding
2	Anal symptoms: itch, pain, burning
3	Change in bowel habit
4	Urinary symptoms (prostate)
5	Incontinence
6	Suspected spinal cord compression — check anal tone

• Normal prostate: rubbery, firm, bilobed, smooth with a central median sulcus. With age it enlarges and the sulcus is lost; cancer is hard and irregular.
• Internal sphincter = smooth muscle (involuntary, on entry). External sphincter = skeletal muscle (voluntary, "squeeze").

Thyroid Examination — OSCE Checklist

Deakin MD2 · ELC5 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Hands → Vitals/Arms → Eyes/Face → Neck → Chest → Legs**. *Look for* **hyper- AND hypothyroid** *signs at every step and say what you do / don't see.*

1. Introduction

☐	**W**ash hands
☐	**I**ntroduce self (name, role)
☐	**P**atient ID: name and DOB
☐	**E**xplain the exam and gain consent
☐	**E**xpose neck to clavicles; **P**osition seated on a chair (room to stand behind)
☐	Ask about pain / neck tenderness; have a glass of water ready

2. Examination

	Step	What to do / say	Abnormal
General inspection			
☐	Body habitus	Signs of weight change	Loss (hyper); gain not necessarily more common (hypo)
☐	Clothing	Dressed for the weather? Heat / cold intolerance	Layers = hypo; sweaty, few clothes = hyper
☐	Well / unwell	Anxiety, agitation, restlessness	Hyper
☐	Obvious neck mass	Goitre or thyroglossal cyst	May be seen before regional exam
Hands			
☐	Nails	Clubbing (thyroid acropachy), onycholysis	Hyper (Graves)
☐	Skin	Palmar erythema, warm, sweaty	Hyper
☐	Skin	Peripheral cyanosis, palmar pallor, yellowing (hypercarotenaemia)	Hypo
☐	Fine tremor	Arms outstretched, sheet of paper on the backs of hands	Hyper (sympathetic overactivity)
Vital signs and arms			
☐	Radial pulse (manual)	Rate, rhythm, volume	Tachycardia / AF (hyper — confirm on ECG); slow, small volume (hypo)
☐	Vital signs	BP, RR, SpO_2, temp	Wide pulse pressure, fever (hyper) †
☐	Arm reflexes	Biceps, brachioradialis	Brisk (hyper); slow relaxation (hypo) †
☐	Proximal myopathy	Arms overhead / resisted "chicken wings" (can do with legs)	Hypo or hyper
Eyes and face			
☐	Face / skin	Pallor, yellowing, thickened skin, alopecia, vitiligo, xanthelasma	Hypo; vitiligo = autoimmune
☐	Periorbital / eyebrows	Periorbital oedema; loss of outer third of eyebrows	Hypo
☐	Tongue / speech	Macroglossia, hoarse or slow speech	Hypo
☐	Exophthalmos	Look from front, side and above/behind	Graves' disease only
☐	Lid retraction / stare	Sclera visible above iris	Hyper
☐	Lid lag	Follow finger from high to low	Hyper (sympathetic, Müller's muscle)
☐	Conjunctiva / cornea	Chemosis, conjunctivitis, corneal ulceration	Graves (can't close eyes)
☐	Eye movements + ptosis	H test, ask about diplopia	Ophthalmoplegia (Graves); ptosis — consider myasthenia
Neck — inspect (front and sides)			
☐	Scars, redness, veins	Thyroidectomy scar; redness (thyroiditis); prominent veins	Retrosternal goitre
☐	Swelling	Note any goitre or mass	Diffuse vs nodular
☐	Swallow	"Take a sip of water and hold it… now swallow"	Goitre and thyroglossal cyst move up
☐	Tongue protrusion	"Stick out your tongue"	Thyroglossal cyst moves up; goitre does not
Neck — palpate (from behind)			
☐	Thyroid	Warn patient; both lobes + isthmus: size, shape, consistency, symmetry, tenderness, nodules	Diffuse (Graves), multinodular, solitary nodule
☐	Swallow	Palpate while patient swallows water	Tethering = possible malignancy
☐	Lymph nodes	Cervical and supraclavicular chains	Malignancy
☐	Trachea*	Position †	Deviation from large goitre
Neck — percuss and auscultate			
☐	Percuss	Over upper sternum	Dullness = retrosternal goitre
☐	Pemberton's sign	Arms up beside ears for 1 min: facial plethora, cyanosis, neck vein distension, stridor	Thoracic inlet obstruction

	Step	What to do / say	Abnormal
☐	Auscultate	Over each lobe with diaphragm for a bruit	Bruit = hyper (Graves)
Chest			
☐	Gynaecomastia	If not already seen	Hyperthyroidism, hypogonadism
☐	Heart / lungs	Auscultate for pericardial and pleural effusions	Hypo
Legs			
☐	Pretibial myxoedema	Shins: raised, waxy, orange-peel plaques	Graves
☐	Oedema	Press over shin	Non-pitting = hypo
☐	Proximal myopathy	Stand from chair, arms crossed	Hypo or hyper
☐	Reflexes	Knee, ankle	Brisk (hyper); slow relaxing (hypo)

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion: "I would check TFTs (TSH, free T4 ± T3), thyroid antibodies, and arrange a thyroid ultrasound if a goitre or nodule is found." †
☐	Present: state thyroid status first, then the neck: "Mrs X appears clinically euthyroid. There is no goitre, the neck is non-tender with no lymphadenopathy, and no eye signs of Graves' disease."

4. Quick reference

Region	Hyperthyroid	Hypothyroid
General	Weight loss, anxious, agitated, heat intolerance	Weight gain, slow, cold intolerance, dressed warmly
Hands	Sweaty, warm, palmar erythema, onycholysis, acropachy, fine tremor	Cool, dry, pale, yellow (carotenaemia), peripheral cyanosis
Pulse / reflexes	Tachycardia, AF; brisk reflexes	Bradycardia, small volume; slow-relaxing reflexes
Eyes / face	Lid retraction, lid lag; Graves: exophthalmos, chemosis, ophthalmoplegia	Periorbital oedema, loss of outer ⅓ eyebrows, coarse thick skin, alopecia
Neck	Goitre ± bruit (Graves)	Goitre (Hashimoto's) or none
Legs	Pretibial myxoedema (Graves), proximal myopathy	Non-pitting oedema, proximal myopathy

- Goitre: 80% euthyroid, 10% hypo, 10% hyper. Isthmus lies over tracheal rings 2–4.
- Only **Graves' disease** causes exophthalmos and pretibial myxoedema; lid lag and stare occur in any hyperthyroidism.
- Pitfalls: palpating from the front only, forgetting the swallow and tongue tests, not commenting on thyroid status, skipping the legs.

Menstrual History — OSCE Checklist

Tick each item as you ask it. **Shaded rows (*)** *are asked if relevant.* † *= added from standard teaching, not in the Deakin guideline. Use sensitive, non-judgemental language and establish the patient's goals and concerns.*

1. Opening

☐	Introduce self, confirm patient ID, explain purpose; consent
☐	Signpost: "Some of these questions are personal — you don't have to answer anything you're not comfortable with."
☐	Open question: "Do you have periods or a menstrual cycle?"

2. The cycle

	Ask about	Example question	Notes
☐	Menarche	"How old were you when your periods started?"	
☐	Menopause*	"Have your periods stopped?"	Menopause = 12 months since the final period; before this = perimenopause
☐	LMP / LNMP	"When did your last normal period start?"	Any chance of pregnancy?
☐	Regularity	"Do they come at about the same time each month?"	Regular vs irregular
☐	Cycle length	"How many days from the start of one period to the start of the next?"	Normal ~21–35 days
☐	Duration	"How many days do you bleed for?"	Normal ~4–7 days (30–80 mL)
☐	Tracking	"Do you track your cycle, e.g. with an app?"	Helps with oligo-/amenorrhoea

3. Problems with periods

	Ask about	Example question	Notes
Heavy menstrual bleeding (HMB) — "Is it too heavy?"			
☐	Clots, flooding, leakage	"Do you pass clots or leak through?"	Suggests HMB
☐	Products	Pads, tampons, cups, period underwear; using two at once?	Dual products = heavy flow
☐	Frequency of change	"How often do you change, and are they soaked?"	Change <2-hourly = significant
☐	Impact	Physical, emotional, social, work/school	Defines HMB
☐	Anaemia symptoms*	Fatigue, breathlessness, dizziness †	Iron deficiency
Dysmenorrhoea — "Is it too painful?"			
☐	Timing and site	When does pain start (may precede bleeding by days)? Where? How long?	
☐	Impact	Missing school or work? What helps?	
☐	Associated symptoms	Nausea, vomiting, diarrhoea, constipation	Consider endometriosis
Cycle too frequent or infrequent			
☐	Oligo- / amenorrhoea	Cycle length; periods stopped? Pregnancy?	Consider PCOS, thyroid, weight change †
Abnormal bleeding — red flags			
☐	Intermenstrual (IMB)	Bleeding between periods?	Cervical / endometrial pathology
☐	Post-coital (PCB)	Bleeding after sex?	Cervical pathology
☐	Post-menopausal (PMB)	**Any** bleeding or spotting after menopause?	Red flag — exclude endometrial cancer urgently

4. Management of periods

☐	Hormonal	COCP, POP, IUD, implant, injection — not always used for contraception	All can alter the cycle
☐	Non-hormonal	NSAIDs, tranexamic acid	Pain and flow control

5. Link to related history and close

☐	Gynaecological, obstetric, sexual health (± gender identity) and surgical history as relevant
☐	Thyroid symptoms* (heat/cold intolerance, weight change) — thyroid disease can change menstrual bleeding
☐	Ideas, concerns, expectations: what does the patient want? (some wish to have, others not to have periods)
☐	Summarise back, check accuracy, invite questions, thank patient

6. Quick reference

Type	Causes of HMB
Structural	Fibroids (leiomyomata), adenomyosis, endometrial polyps, endometrial hyperplasia, endometrial cancer
Non-structural	Ovulatory disorders (PCOS, perimenopause, puberty), clotting disorders, iatrogenic (IUD), hypothyroidism
Mnemonic †	PALM-COEIN

Term	Meaning
HMB	Excess loss interfering with quality of life
IMB	Bleeding between periods
PCB	Bleeding after intercourse
PMB	Any bleeding ≥12 months after the final period
Cycle length	Day 1 of one period to day 1 of the next

Diabetes Examination — OSCE Checklist

Deakin MD2 · ELC3 · Time: 10–12 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Vitals → Hands → Face → Eyes → Neck/Chest → Abdomen → Lower limbs**. *Look for* **microvascular** *(eyes, kidneys, nerves) and* **macrovascular** *(heart, carotids, legs) complications.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID
☐	Explain and gain consent; ask about pain
☐	Position and expose as for CV exam, plus abdomen and lower limbs (keep the patient covered and comfortable)

2. Examination

	Step	What to do / say	Significance
General inspection			
☐	Age, body habitus	Obesity, central adiposity; cachexia	T2DM, metabolic syndrome
☐	Acute distress	Hyperventilating, unwell (Kussmaul breathing); confused, anxious, sweaty	DKA; hypoglycaemia
☐	Gait	Observe as the patient enters	Neuropathy, PVD, foot problems
Vital signs and measurements			
☐	BP lying and standing	Postural drop	Autonomic neuropathy, dehydration
☐	HR, RR, SpO_2, temp	Full set	Sepsis, DKA
☐	BGL ± ketones	Ketones if BGL elevated	Ketones >0.6 mmol/L = red flag
☐	Weight, height, BMI, waist	If appropriate and able	Cardiometabolic risk
Hands and arms			
☐	Fingertips	Finger-prick marks; sites rotated?	Self-monitoring
☐	Dexterity	OA, limited joint mobility	Ability to self-inject / test
☐	Upper arm	Continuous glucose monitor (CGM)?	
Face			
☐	Endocrine facies	"Moon face"	Cushing's (secondary diabetes)
☐	Mouth	Infection: bacterial or Candida (white plaques)	Poor glycaemic control
Eyes			
☐	Visual acuity	Snellen chart, glasses on; each eye	Retinopathy, maculopathy, cataract
☐	Eye movements*	If clinically indicated	CN III / VI mononeuropathy
☐	Fundoscopy	Red reflex, disc, vessels, macula (note undilated-pupil limits)	Retinopathy, cataract
Neck and chest			
☐	Carotids	Auscultate for bruits	Carotid atherosclerosis
☐	Acanthosis nigricans	Brown velvety skin: back of neck, axillae, folds	Insulin resistance
☐	Precordium	Auscultate heart; ± lung bases if cardiac concern	Heart failure, IHD
Abdomen			
☐	Injection sites	Clean? Rotated? Lipohypertrophy or lipoatrophy	Erratic insulin absorption
Lower limbs — inspect (incl. backs of legs, between toes, soles)			
☐	Gait and footwear	Gait; shoe type and fit	Pressure areas
☐	Nails and foot care	Nail condition; impingement on adjacent toes	
☐	Interdigital	Maceration, tinea pedis	Infection portal
☐	Skin	Dry fissured skin, atrophy, callus, necrobiosis lipoidica	Neuropathy, high pressure
☐	Pressure areas, ulcers	Heels, metatarsal heads, toes: ulcer, crack, erythema; depth?	Ulcer to tendon/bone = red flag
☐	Trophic changes	Hair loss, shiny skin	PVD
☐	Bone architecture	High arch, clawed toes, bunions, muscle wasting, Charcot joint	Motor / sensory neuropathy
Lower limbs — vascular			
☐	Pulses	Distal → proximal: DP, PT, popliteal, femoral	PVD
☐	Perfusion	Capillary refill in toes; temperature of limbs	Critical limb ischaemia = red flag
Lower limbs — neurological			
☐	Monofilament (10 g)	Eyes closed; several plantar sites each foot (hallux, 1st/3rd/5th met heads †)	Loss of protective sensation
☐	Vibration	128 Hz fork at base of great toe; move proximally if absent	Dorsal column / large fibre — most sensitive
☐	Proprioception	Great toe, held by its sides; move proximally if absent	Sensory ataxia
☐	Reflexes	Plantar → ankle → knee	Reduced ankle jerks in neuropathy

3. Closing and presenting

☐	Thank patient, help them dress, wash hands
☐	Completion: "I would perform urinalysis (ketones, leucocytes, nitrites ± albumin), and check HbA1c, UEC/eGFR, urine ACR and lipids †."
☐	Present by complication: glycaemic control clues → microvascular (eyes, nerves, kidneys) → macrovascular (carotids, heart, peripheral pulses) → feet

4. Quick reference

Complication	Examination findings
Retinopathy	Microaneurysms, dot-blot haemorrhages, hard exudates → cotton-wool spots → new vessels †
Neuropathy	Stocking sensory loss, ↓ vibration, absent ankle jerks, Charcot foot
Nephropathy	Oedema, ↑ BP; needs urine ACR †
Macrovascular	Carotid bruit, absent pulses, trophic changes, ↓ cap refill

#	Red flags (Deakin guideline)
1	Symptomatic hyperglycaemia ± dehydration
2	Ketosis >0.6 mmol/L or ketoacidosis
3	Severe infection / sepsis
4	Ulcer with depth to tendon or bone
5	Critical limb ischaemia
6	Pregnancy

Breast Examination — OSCE Checklist

Deakin MD2 · ELC7 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Introduction → Inspection (seated) → Axillae + supraclavicular (seated) → Breast palpation (supine) → Close**. *Examine* **both breasts and draining nodes**. *Communicate throughout in your own words.*

1. Introduction (intimate examination)

☐	**Introduce** self (name, role); confirm patient ID
☐	**Explain** what the exam involves and why; check understanding
☐	**Offer a chaperone / observer** (document if declined)
☐	**Consent**: verbal, informed; "Just say stop and I will"
☐	**Wash hands**; gloves where indicated
☐	**Privacy**: undress behind curtain, drape/cover, warm room, good light
☐	Ask about symptoms, noticed changes, past surgery or radiotherapy
☐	Position: seated on the couch; drape so one breast/axilla is exposed while the other is covered

2. Inspection — seated, arms by sides

	Inspect for	What to say / significance
☐	Introduce step	"I'm going to start by looking at your breasts. Could you sit up straight and lower your gown when you're ready?"
☐	Size, shape, symmetry	Asymmetry may be normal — compare with patient's baseline
☐	Lumps or thickenings	Visible contour change
☐	Skin puckering / dimpling	Tethering by underlying tumour
☐	Peau d'orange	Pitted oedematous skin — lymphatic obstruction
☐	Erythema, prominent veins	Inflammation, infection, inflammatory carcinoma
☐	Sores or ulcers	On breast or nipple
☐	Nipple retraction	New drawing-in of the nipple
☐	Rash on / around nipple	Eczematous change — think Paget's disease of the nipple
☐	Axilla and upper arm	Axillary swelling or lumps; arm swelling (lymphoedema)
☐	Pendulous breasts*	Ask patient to lift breasts to inspect underneath

3. Inspection with movement

	Manoeuvre	Look for
☐	"Could you follow my arm movements?" — raise arms above head	Tethering: tissue, skin or nipple not moving smoothly or symmetrically
☐	Hands on hips, lean forward and press hands into hips (pectoral contraction)	Increased dimpling = fixation to chest wall

4. Palpation — lymph nodes (seated)

☐	Hold the patient's outstretched arm with yours to relax the axilla; palpate with your free hand (gloves may be preferred)
☐	Palpate all 5 walls: **anterior, posterior, medial, lateral, apex**
☐	Repeat on the other axilla — always examine both
☐	Palpate supraclavicular fossae both sides
☐	"Thank you. Pull the gown up, and when you're ready, please lie down."

5. Palpation — breasts (supine)

☐	Uncover **one breast at a time**; patient's arm abducted with hand behind head
☐	Flat pulps of fingers, small circles: light pressure then deeper, before moving on (other hand may support tissue)
☐	Cover the whole area: clavicle → sternum → mid-axillary line → bra line
☐	Systematic pattern: vertical strips ("lawn mowing") or spiral out from the nipple; minimise lifting hand off skin
☐	Palpate behind the nipple for lumps
☐	Include the **axillary tail** (superolateral)
☐	Nipple discharge*: ask patient to express if discharge reported; note colour, single/multiple ducts †
☐	Repeat fully on the other breast

6. If you find a lump — describe it

☐	Position	Quadrant, clock-face position and distance from areola
☐	Size	Diameter in cm (or pea / grape / plum)
☐	Shape and surface	Rounded, regular / irregular, smooth / rough
☐	Consistency	Fluctuant, soft, firm, hard
☐	Tenderness	How tender
☐	Mobility	Mobile or fixed — to skin or deep (chest wall)
☐	Borders	Discrete or poorly defined
☐	Retraction	Altered contour, dimpling
☐	Number	Single or multiple; check the other breast

7. Closing

☐	Thank patient; let them know they can dress; give privacy
☐	Explain findings and next steps; invite questions
☐	Document consent, chaperone, examination, findings and discussion
☐	Present: "On inspection the breasts were symmetrical with no skin or nipple changes. No masses on palpation of either breast or the axillary tails, and no axillary or supraclavicular lymphadenopathy. This is a normal breast examination."

8. Quick reference

Feature	Fibroadenoma †	Cyst †	Carcinoma †
Age	15–35	35–55, perimenopausal	Usually >40
Feel	Smooth, rubbery	Smooth, fluctuant / firm	Hard, irregular
Mobility	Very mobile ("breast mouse")	Mobile	Often fixed
Skin / nipple	Normal	Normal	Dimpling, peau d'orange, retraction

Key fact	From the Deakin guideline
Triple test	Clinical exam + imaging (mammogram/US; MRI gold standard) + non-excisional biopsy for **any** lump
BreastScreen	Free mammogram 2-yearly from 40; actively invited 50–74
Burden	Most common cancer in Australian females; 1 in 8 by age 85
Lymph drainage	Lateral/central → axillary nodes; medial → internal mammary (not palpable)
Commonest site	Upper outer quadrant †

- Breast examination alone is **not** recommended as a screening test (RACGP) — use it to teach breast awareness.
- Report promptly: new lump, nipple change or discharge, skin colour/texture change, pain, swelling.
- Pitfalls: exposing both breasts at once, missing the axillary tail or behind the nipple, forgetting supraclavicular nodes, no chaperone offer.

Inguinoscrotal Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Introduction → Standing: inspect, palpate, special tests → Supine: re-examine, reduce, transilluminate, percuss, auscultate**. *Not assessed practically in Year 2, but assessable in written form.*

1. Introduction (intimate examination)

☐	**Introduce** self (name, role); confirm patient ID
☐	**Explain** what the exam involves and why; check understanding
☐	**Offer a chaperone / observer** (document if declined)
☐	**Consent**: verbal, informed; "Just say stop and I will"
☐	**Wash hands**; gloves where indicated
☐	**Privacy**: undress behind curtain, drape/cover, warm room, good light
☐	Sheet provided; **gloves on both hands** for the whole examination
☐	Sign outside the cubicle so you are not interrupted
☐	Start with patient **standing**; undress lower garments behind curtain (gown if preferred)
☐	Ask patient to show you the area of concern; ensure adequate exposure (umbilicus to mid-thigh)

2. Inspection — standing

	Inspect	Look for
☐	Skin of lower abdomen, inguinal regions, upper inner thighs, scrotum	Scars, erythema, rashes
☐	Swellings / lumps	Groin or scrotal; note side
☐	Testicular asymmetry	Left usually hangs lower than right (normal)
☐	Scrotal oedema	Note extent
☐	Cough impulse — visible	"Please turn your head and cough": look for bulge each side. No impulse in obstructed, strangulated or omental hernia

3. Palpation — inguinal region (standing)

☐	Compare both sides: palpate from pubic tubercle to ASIS, above and below the inguinal ligament
☐	Inguinal lymph nodes; femoral artery pulse
☐	Any swelling: warmth, tenderness, consistency
☐	Extent: can you reach its top and bottom? Does it descend into the scrotum?
☐	Relation to **pubic tubercle** and inguinal ligament: above-medial = inguinal; below-lateral = femoral †

4. Palpation — scrotum (standing)

☐	Warn patient; ask them to say if there is any pain
☐	Palpate one testis at a time between thumb and fingertips (or cradle with middle + ring fingers, thumb above)
☐	Both testes present? Estimate size of each; compare
☐	Palpate testis, epididymis (posterior) and spermatic cord
☐	Warmth, tenderness, swellings or lumps; consistency
☐	Can you **get above** the swelling? Gently pinch the upper scrotum between thumb and fingers

5. Special tests (standing)

	Test	How	Interpretation
☐	Cough impulse — palpated	Fingers over pubic tubercle region; patient coughs; each side	Expansile impulse = hernia
☐	Deep ring occlusion	Reduce, then press over deep ring (midpoint of inguinal ligament †); cough; each side	Controlled = indirect; still protrudes = direct
☐	Cremasteric reflex*	Stroke upper inner thigh	Testis rises; lost in torsion

6. Supine

	Step	How	Interpretation
☐	Re-examine	Re-palpate any swelling found standing	Exact site and nature clearer supine
☐	Reducibility	Attempt gentle manual reduction; feel for gurgling	Irreducible = incarcerated; tender + irreducible = strangulation
☐	Transillumination	Darkened room, torch against scrotal swelling	Lights up = cystic; dark = solid
☐	Percussion	Percuss over swelling	Hollow = gas-filled bowel; splash = obstructed loop
☐	Auscultation	Over the mass and the abdomen	Bowel sounds = bowel in hernia

7. Closing

☐	Check for undue pain or distress
☐	Privacy to dress
☐	Explain findings and next steps; invite questions
☐	Restore examination area/couch
☐	Document consent, chaperone, examination, findings and discussion
☐	Completion statement: "I would also examine the abdomen and external genitalia †"

8. Quick reference

Hernia †	Site	Deep ring occlusion	Into scrotum?	Other
Indirect inguinal	Through deep ring, lateral to inferior epigastric vessels	Controlled	Often	Most common; younger men
Direct inguinal	Hesselbach's triangle, medial to inferior epigastric vessels	Not controlled	Rarely	Older; weak posterior wall
Femoral	Below and lateral to pubic tubercle	—	No	More common in women; high strangulation risk

Scrotal lump †	Get above?	Separate from testis?	Transilluminates?	Clues
Inguinoscrotal hernia	No	Yes	No	Cough impulse, may reduce
Hydrocele	Yes	No — testis within	Yes	Smooth, painless
Epididymal cyst	Yes	Yes (above / behind)	Yes	Small, smooth
Varicocele	Yes	Yes	No	"Bag of worms", left, empties lying down
Testicular tumour	Yes	No — part of testis	No	Hard, painless, irregular; urgent US
Torsion / epididymo-orchitis	Yes	—	No	Torsion: acute pain, high-riding, absent cremasteric — surgical emergency

Cervical Screening Test (Speculum) — OSCE Checklist

Deakin MD2 · ELC8 · Clinician-collected CST

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Prepare and consent → Equipment → Position → Inspect → Speculum → Sample → Remove → Close**. *Practised on part-task trainers — speak to the model as you would to a patient.*

1. Before the patient is on the couch

☐	Why a pelvic examination is needed; what it involves (speculum, CST, swabs, bimanual) and the position
☐	Past experience of pelvic examinations
☐	Obstetric history: children? vaginal births? (guides speculum size)
☐	Menstrual history: LMP; possibility of pregnancy
☐	Verbal consent; "Just say stop and I will"
☐	Offer a chaperone (advocate for patient and doctor; often a nurse who assists)
☐	Offer to empty bladder first
☐	Timing*: ideally ~2 weeks after day 1 of last period (if menstruating)

2. Equipment

☐	Gloves	Cushion / folded blanket to raise pelvis	Manoeuvrable light
☐	Lubricant	Range of speculum sizes	Swabs if needed
☐	Cytobrush + liquid-based vial (ThinPrep)	Single-use mat (bluey)	Sanitary pad and tissues
☐	Chaperone present	Vial labelled and form completed †	

3. Procedure

☐	Gather equipment; hand hygiene; gloves
☐	Patient undresses from waist down behind curtain and covers with blanket
☐	Enter with permission
☐	Position: supine, knees drawn up, heels together, let knees fall apart (do **not** push knees apart)
☐	Expose waist down only when ready; adjust light
☐	Inspect external genitalia: vulval skin, discharge, lesions, prolapse †
☐	Warn the patient; part labia with non-dominant hand
☐	Insert lubricated speculum with blades sideways, rotating during insertion as patient breathes out; aim towards sacrum, not head
☐	Gently open blades until cervix is visualised
☐	Fix speculum open; inspect cervix: os, ectropion, polyps, discharge, lesions †
☐	Insert cytobrush into the os and rotate 2–3 times
☐	Remove brush and agitate well in the ThinPrep liquid
☐	Loosen speculum and remove gently as patient breathes out (watch vaginal walls †)
☐	Cover patient with blanket; tell them the procedure is complete
☐	Privacy to dress; tissues and bin available

4. Closing

☐	Explain how and when results will be received; follow-up plan
☐	Invite questions; check comfort
☐	Document consent, chaperone, findings, tests taken, follow-up

5. Quick reference

- Bimanual examination is **not** recommended routinely in asymptomatic people attending for screening.
- Australian NCSP: HPV-based CST every 5 years for people with a cervix aged 25–74; self-collection is an option for everyone †.
- Clinician collection (HPV + liquid-based cytology) is required for symptoms (PCB, IMB, PMB, persistent unexplained discharge) and for AIS surveillance.
- Over 70% of Australians diagnosed with cervical cancer were under-screened or never screened.
- Pitfalls: inserting blades vertically, aiming upwards, opening blades before full insertion, closing blades on the cervix, exposing the patient before they're ready.

Bimanual Pelvic Examination — OSCE Checklist

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Prepare and consent → Position → Inspect → Internal palpation → Bimanual → Close**. *Indicated for genitourinary symptoms and in pregnancy —* **not** *as routine screening.*

1. Before the patient is on the couch

☐	Why a pelvic examination is needed; what it involves (speculum, CST, swabs, bimanual) and the position
☐	Past experience of pelvic examinations
☐	Obstetric history: children? vaginal births? (guides speculum size)
☐	Menstrual history: LMP; possibility of pregnancy
☐	Verbal consent; "Just say stop and I will"
☐	Offer a chaperone (advocate for patient and doctor; often a nurse who assists)
☐	Offer to empty bladder first

2. Procedure

☐	Gather equipment (gloves, lubricant, light, bluey, tissues); hand hygiene; gloves
☐	Patient undresses from waist down behind curtain, covers with blanket; enter with permission
☐	Position: supine, knees drawn up, heels together, knees fall apart
☐	Expose waist down; inspect external genitalia
☐	Warn patient; gently touch the introitus, then slowly insert 2 lubricated gloved fingers
☐	Palpate cervix: position, consistency, os, tenderness †
☐	**Cervical excitation**: gently move the cervix side to side — watch the face for pain
☐	Uterus: other hand on lower abdomen pressing down towards internal fingers — size, shape, position (anteverted / retroverted), mobility, tenderness †
☐	Adnexa: internal fingers in each lateral fornix, abdominal hand over iliac fossa — masses, tenderness; both sides
☐	Withdraw fingers; check glove for blood or discharge
☐	Cover patient; leave tissues and bin; privacy to dress

3. Closing

☐	Explain findings and next steps; invite questions
☐	Document consent, chaperone, findings, investigations, treatment and follow-up

4. Quick reference †

Finding	Think of
Cervical excitation (motion tenderness)	Pelvic inflammatory disease, ectopic pregnancy
Enlarged, irregular, firm uterus	Fibroids; pregnancy (smooth, soft)
Bulky, tender uterus	Adenomyosis
Fixed, retroverted, tender uterus ± nodules	Endometriosis
Adnexal mass	Ovarian cyst or tumour, ectopic, tubo-ovarian abscess
Blood on glove	Menses, IMB/PMB — correlate with history

- Never uncover the patient yourself if avoidable — ask them to do so when comfortable.
- Pitfalls: no chaperone offer, not warning before touching, examining without watching the patient's face, not checking the glove.

HPV Self-Collection Discussion — OSCE Checklist

Deakin MD2 · ELC8 · Counselling station

Tick each point as you cover it. **Shaded rows (*)** *are covered if relevant.* † *= added from standard teaching, not in the Deakin guideline. Use plain language and check understanding.*

1. Opening

☐	Introduce self, confirm patient ID, explain purpose of the discussion
☐	Ask what the patient knows about cervical screening; screening history (last test, results)
☐	Explore concerns (past experiences, cultural or personal preferences)

2. Check eligibility

	Check	Ask about	If yes
☐	Offer to all	Self-collection is an option for anyone in routine cervical screening	Highly acceptable to under- and never-screened people
☐	Symptoms?	Post-coital, intermenstrual or post-menopausal bleeding; persistent unexplained discharge	Contraindicated → clinician-collected HPV + cytology
☐	Surveillance?	Follow-up after completely excised adenocarcinoma in situ (AIS)	Contraindicated → clinician collection

3. Explain why it's worthwhile

☐	As sensitive as a clinician-collected test; easy to do
☐	Over 70% of Australians diagnosed with cervical cancer were under-screened or never screened
☐	Priority groups*: CALD communities, people with disability, LGBTQIA+ people, rural and remote, Aboriginal and Torres Strait Islander people

4. Teach the technique

☐	Wash hands; find a private, comfortable position †
☐	Twist the cap and remove the swab
☐	Insert swab into the vagina about the length of an index finger
☐	Rotate for at least **10 seconds**
☐	Place swab back in the tube and screw the cap on
☐	Do **not** put the swab down on any surface between opening and closing
☐	Return the sample to the healthcare provider

5. Results and close

☐	Sample goes to a pathology lab for HPV testing; results go to the healthcare provider
☐	Explain follow-up: HPV not detected → rescreen in 5 years; HPV detected → may need a clinician-collected sample or colposcopy †
☐	Teach-back: ask the patient to explain the steps; invite questions; thank patient
☐	Document discussion and plan

Shoulder Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *Examine the normal side first if one side is painful †.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: usually start standing
☐	Exposure: sleeveless top at least (singlet ideal); compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both sides, from front, side and back			
☐	Posture	Shoulder height, position of arm	Guarding, dislocation
☐	Skin and scars	Erythema, bruising, arthroscopy or surgical scars	Infection, previous surgery
☐	Swelling	Over joint, AC joint	Effusion, AC injury
☐	Wasting	Deltoid, supraspinatus and infraspinatus fossae	Rotator cuff tear, nerve injury
☐	Deformity	Step at AC joint, squared-off shoulder, winged scapula †	AC dislocation, glenohumeral dislocation
Feel — start medially and work round to the back			
☐	Temperature	Back of hand over both shoulders	Warm = inflammation, infection
☐	SC joint → clavicle → AC joint	Palpate along the bones	Tenderness, step, swelling
☐	Head of humerus	Anterior and lateral	Tenderness
☐	Long head of biceps	Bicipital groove (arm externally rotated †)	Biceps tendinopathy
☐	Scapula	Borders and spine	Tenderness
☐	Crepitus	Hand over joint while moving it through a limited range	OA, rotator cuff disease
Move — active, then passive if limited (watch from front and back)			
☐	Abduction	"Lift your arms out to the side and up" (0–180°); palpate for crepitus	Painful arc 60–120° = impingement †
☐	Adduction	"Bring your arm across your body"	
☐	Flexion	"Lift your arms forwards and up" (0–180°)	
☐	Extension	"Move your arms backwards" (0–60°)	
☐	Internal rotation	Hand behind back, reach up the spine (note highest level †)	Frozen shoulder, cuff
☐	External rotation	Elbows tucked in at 90°, rotate forearms out (0–60°)	Lost first in frozen shoulder †
☐	Measure	Limited → estimate/measure degrees; otherwise "full range of motion"	
Special tests			
☐	Empty can (Jobe's)	Arm at 90° elevation, 30° forward (scapular plane), thumbs down; push down while patient resists	Pain / weakness = supraspinatus (rotator cuff)
☐	Hawkins (impingement)	Shoulder and elbow flexed to 90°; your arm as fulcrum, internally rotate the shoulder	Pain = subacromial impingement; negative = unlikely
☐	Apprehension (supine)	Shoulder 90° abducted, elbow 90° flexed; gently externally rotate	Pain or apprehension = anterior instability
☐	Clunk test	Supine, arm fully abducted; full external rotation, push humeral head slightly anteriorly	Grinding / clunk = labral tear †
☐	Sulcus sign	Arm at side, neutral rotation; pull down at the elbow	Gap >1 fingerbreadth below acromion = inferior instability

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the neck and elbow (joints above and below) and do a neurovascular exam of the upper limb †."
☐	Present: "On inspection there was no wasting, swelling or deformity. No tenderness or warmth. Full active and passive range of motion without pain. Special tests for rotator cuff, impingement and instability were negative."

4. Quick reference †

Pattern	Key findings
Rotator cuff tear / tendinopathy	Weak, painful abduction; positive empty can; wasting of supra/infraspinatus
Subacromial impingement	Painful arc 60–120°; positive Hawkins
Frozen shoulder (adhesive capsulitis)	Global loss of active AND passive movement, external rotation first
Anterior instability	Previous dislocation; positive apprehension
AC joint pathology	Tenderness/step at AC joint; painful arc at the top (170–180°)

Elbow Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 6–8 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests**.

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: usually start standing
☐	Exposure: sleeveless top at least; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both sides, from front, side and back			
☐	Carrying angle	Arms by sides, palms forward	Cubitus valgus / varus (old fracture)
☐	Swelling	Over the joint and olecranon	Effusion, olecranon bursitis
☐	Skin	Erythema, scars, psoriatic plaques †	
☐	Nodules	Rheumatoid nodules, gouty tophi (extensor surface)	RA, gout
☐	Wasting	Forearm muscles	
Feel			
☐	Temperature	Back of hand, compare sides	Warm = inflammation, infection
☐	Lateral epicondyle	Palpate	Tender = tennis elbow
☐	Medial epicondyle	Palpate	Tender = golfer's elbow
☐	Olecranon tip	Palpate	Bursitis, fracture
☐	Discrete swellings	Nodules, bursa	
☐	Effusion	Thumb along edge of ulna just distal to olecranon; fully extend the elbow	Palpable bulge = fluid
Move — active, then passive if limited			
☐	Flexion	"Bend your elbow" (0–150°)	
☐	Extension	"Straighten your elbow" (0°)	Loss of extension = effusion, OA
☐	Pronation	Elbows tucked in at 90°: "Turn your palms down" (0–80°)	
☐	Supination	"Turn your palms up" (0–80°)	
☐	Crepitus	Feel during passive movement	OA
☐	Measure	Limited → degrees; otherwise "full range of motion"	
☐	Ulnar nerve*	If deformity or numbness: neuro exam of the hand and arm	Ulnar nerve entrapment
Special tests			
☐	Lateral epicondylitis	Elbow 90°, forearm pronated, wrist flexed; support elbow; patient extends wrist against resistance	Pain at lateral epicondyle = tennis elbow
☐	Medial epicondylitis	Elbow 90°, forearm supinated; support elbow; patient flexes wrist against resistance	Pain at medial epicondyle = golfer's elbow

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the shoulder and wrist (joints above and below) and do a neurovascular exam of the upper limb †."
☐	Present: "Normal carrying angle with no swelling, nodules or scars. No tenderness over the epicondyles or olecranon and no effusion. Full range of flexion, extension, pronation and supination. Tests for lateral and medial epicondylitis were negative."

Hand & Wrist Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *Includes* **Measure** *(function) for the hand.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: seated on bed (or at a table), hands resting **palms down on a pillow**
☐	Exposure: hands and arms to the elbows
☐	Ask about pain before touching
☐	General observation: systemically well? in pain? age; other joint or skin disease (psoriasis); walking aids, splints

2. Examination

	Step	What to do / say	Abnormal
Look — forearms, wrists and hands (dorsal, then palms †)			
☐	Distribution	Symmetry, which joints are affected	Symmetrical small joints = RA; DIPs = OA, psoriatic
☐	Skin	Erythema, atrophy, rashes, scars (carpal tunnel or tendon release)	Psoriasis, previous surgery
☐	Nails	Pitting, onycholysis	Psoriatic arthritis
☐	Swelling	Joint swelling; "sausage" fingers	Synovitis; dactylitis (psoriatic)
☐	Deformity	Ulnar deviation, swan neck, boutonnière, Z-thumb; Heberden's/Bouchard's nodes †	RA; OA
☐	Wasting	Thenar or hypothenar eminence, dorsal interossei	Median / ulnar nerve lesion
☐	Function	Watch how they use their hands informally	
Feel			
☐	Temperature	Back of hand over wrists and joints	Warm = active inflammation
☐	Wrists	Palpate with both thumbs	Tenderness, synovitis
☐	MCP joints	Squeeze across MCPs gently, then palpate each †	Tenderness = synovitis (early RA)
☐	PIP and DIP joints	Palpate each joint	Boggy (synovitis) vs hard bony (OA)
☐	Joint features	Tenderness, synovitis/effusion, warmth, subluxation	
☐	Scaphoid*	After trauma: anatomical snuffbox and scaphoid tubercle (thenar)	Scaphoid fracture
☐	Pulses	Radial and ulnar pulses; capillary refill	Vascular compromise
Move — active			
☐	Wrist	Prayer and reverse prayer: flexion and extension †	Reduced range
☐	Fist and straighten	"Make a fist, then straighten your fingers"	Screen MCP/IP movement
☐	Thumb opposition	Touch thumb to each fingertip	
Move — passive			
☐	Wrists, MCP, PIP, DIP	Move each through range; feel for crepitus	Restriction, crepitus
Measure — hand function			
☐	Grip strength	"Squeeze my fingers"	Weak grip
☐	Key grip	Use lock and key to open lock	
☐	Opposition strength	Thumb to each finger, try to pull apart	Median nerve weakness
☐	Practical tasks	Undo screw-top jar; do up and undo a button; write with a pen	Functional impact
Special tests			
☐	Phalen's	Hold wrists fully flexed (backs of hands together) for 60 s †	Tingling in median distribution = carpal tunnel
☐	Tinel's	Tap over the flexor retinaculum (carpal tunnel)	Reproduces symptoms (less reliable)
☐	Neuro screen*	Sensory islands and key muscles of median, ulnar and radial nerves (covered in B&B)	Nerve injury — document sensation, movement, circulation after trauma

3. Closing

☐	Check the elbows: rheumatoid nodules, gouty tophi, psoriatic plaques
☐	Thank patient; wash hands
☐	Present: "There is no deformity, swelling or wasting, and the skin and nails are normal. No tenderness or synovitis. Full active and passive range of movement; grip, key grip and opposition are normal and function is preserved. Phalen's and Tinel's were negative."

4. Quick reference †

Feature	Rheumatoid arthritis	Osteoarthritis	Psoriatic arthritis
Joints	Symmetrical MCPs, PIPs, wrists; DIPs spared	DIPs, PIPs, thumb base (1st CMC)	DIPs, asymmetrical; dactylitis
Swelling	Soft, boggy synovitis	Hard, bony	Sausage digits
Deformities	Ulnar deviation, swan neck, boutonnière, Z-thumb, MCP subluxation	Heberden's (DIP) and Bouchard's (PIP) nodes, squaring of thumb base	Telescoping (arthritis mutilans)
Other	Rheumatoid nodules, wasting of small muscles	Crepitus	Nail pitting, onycholysis, plaques

Hip Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: underwear ideally (shorts for comfort); shirt lifted to waist; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — standing			
☐	Walking in	Walking stick or other aid	
☐	Gait	Normal? Antalgic (short stance on painful side)? Trendelenburg (waddling) †	Hip pain, abductor weakness
☐	Inspect front, back, sides	Deformity, scars/wounds, gluteal and quadriceps wasting, symmetry	Previous surgery, disuse
☐	Trendelenburg test	Stand on one leg (hold your hands †); watch iliac crests; repeat on the other leg	Pelvis drops on the lifted side = weak abductors on the **standing** side
Look — supine			
☐	Re-inspect	Anything missed; leg rotation or shortening †	Fractured NOF: shortened, externally rotated
Feel			
☐	Hip joint	Just distal to midpoint of inguinal ligament; watch the face	Joint tenderness
☐	Greater trochanters	Palpate both	Trochanteric bursitis
Move — supine (active, then passive if pain or restriction)			
☐	Flexion	Straight leg raise (to 90°); knee bent (to 120°)	
☐	Thomas test	Hand under lumbar spine; flex one hip fully until back is flat; watch the other leg	Other leg lifts = fixed flexion deformity
☐	Internal / external rotation	Hip and knee at 90°, rotate foot out (internal) and in (external) †	Internal rotation lost first in OA †
☐	Abduction / adduction	Stabilise the pelvis with your other hand on the opposite ASIS †	
☐	Extension	Patient prone: lift leg off bed	
Measure			
☐	True leg length	ASIS → medial malleolus, both sides	Difference = bone shortening (fracture, congenital, surgery)
☐	Apparent leg length	Umbilicus → medial malleolus, both sides	Difference = pelvic tilt, adduction/abduction, spinal curvature
Special tests (confirm done)			
☐	Trendelenburg, Thomas, leg lengths	Usually completed in Look / Move / Measure	

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the lumbar spine and knee (joints above and below) and do a neurovascular exam of the lower limb †."
☐	Present: "Normal gait with no aids. No deformity, wasting or scars; Trendelenburg negative. No tenderness. Full range of movement; Thomas test negative; true and apparent leg lengths equal."

Knee Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: underwear ideally (shorts for comfort); see the whole limb and quadriceps; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — standing			
☐	Inspect front, back, sides	Deformity, scars/wounds, quadriceps wasting, swelling/effusion, symmetry	Effusion, previous surgery
☐	Alignment	Valgus / varus; hyperextension	OA (varus common †)
☐	Popliteal fossa	Baker's cyst	OA, RA
☐	Gait	Normal? Antalgic?	
Look — supine			
☐	Re-inspect	Anything missed	
Feel			
☐	Temperature	Compare both knees	Warm = inflammation, infection
☐	Palpate	Patella, joint lines (knee flexed †), tibial tuberosity, collateral ligaments; watch the face	Joint-line tenderness = meniscus
☐	Bulge / sweep test	Knee extended; hand on suprapatellar pouch; sweep medial side up, then lateral side down — watch medial side †	Bulge = small effusion (preferred test)
☐	Patellar tap	Knee extended; hand above patella pushing fluid down; two fingers press patella	Tap = large effusion
Move			
☐	Active flexion / extension	"Bend your knee… straighten it" (0–140° †)	Limited → measure degrees
☐	Passive	Feel for crepitus through flexion and extension	OA, patellofemoral
Special tests — cruciates			
☐	Posterior sag	Both knees flexed to 90°, look from the side	Tibia sags back = PCL tear
☐	Anterior drawer	Knee 90°, sit gently on foot, grasp tibia, pull forward	>6 mm translation = ACL tear
☐	Posterior drawer	As above, push tibia back	Excess backward movement = PCL tear
☐	Lachman's	Knee 20–30°; hold distal thigh, pull proximal tibia forward	Translation / soft end point = ACL
Special tests — collaterals (knee at 30°)			
☐	MCL	Valgus stress (push knee inwards)	Medial gaping = MCL tear/laxity
☐	LCL	Varus stress (push knee outwards)	Lateral gaping = LCL tear/laxity
Special tests — menisci			
☐	McMurray's — medial	Full flexion; external rotation of tibia + valgus stress, then extend; palpate joint line	Pain, click, locking = medial meniscus
☐	McMurray's — lateral	Full flexion; internal rotation + varus stress, then extend	Pain, click, locking = lateral meniscus
☐	Apley's grind	Prone, knee 90°, stabilise thigh; press down on foot and rotate	Pain or clicking = meniscus
Special tests — patella			
☐	Patellar apprehension	Knee extended; push patella laterally while flexing the knee	Apprehension = patellar instability

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the hip and ankle (joints above and below) and do a neurovascular exam of the lower limb †."
☐	Present: "Normal gait; no deformity, wasting or effusion. No warmth or tenderness. Full range of movement without crepitus. Cruciate, collateral, meniscal and patellar tests were all negative."

Ankle & Foot Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: legs exposed to the knees; shoes and socks off; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — both limbs, front, side and back			
☐	Gait	Normal? Antalgic?	
☐	Swelling, scars, deformity	Ankle and foot	
☐	Wasting	Calf, small foot muscles	
☐	Nails	Psoriatic changes	Psoriatic arthritis
☐	Arches	Transverse (under MTPs) and longitudinal (1st MTP to heel)	Flattened in RA (pes planus)
☐	Calluses	Soles	Abnormal pressure
☐	Forefoot	Hallux valgus, clawing, crowding of toes, sausage toes	Bunion; RA; dactylitis
☐	Footwear*	Wear pattern on shoes †	
Feel			
☐	Temperature	Compare both sides	Warm = inflammation, gout, infection
☐	Ankle joint line	Swelling, tenderness	
☐	Malleoli and ligaments	Both malleoli; medial and lateral ligaments	Fracture, sprain
☐	Base of 5th metatarsal	Palpate	Avulsion fracture
☐	Achilles tendon	Nodules, tenderness, gap †	Tendinitis, rupture, RA nodules
☐	MTP squeeze	Compress 1st and 5th metatarsals together between thumb and finger	Tender = MTP synovitis (early RA)
☐	MTP and IP joints	Palpate individually	Tenderness, swelling; 1st MTP = gout †
Move			
☐	Ankle	Dorsiflexion and plantarflexion (active, then passive)	
☐	Subtalar joint	Inversion and eversion (hold the heel †)	
☐	MTP and IP joints	Passive flexion and extension of each	Hallux rigidus †
Special tests			
☐	Knee-to-wall	Heel flat, bend knee to touch wall; move foot back until heel lifts or pain; measure great toe to wall	Normal ≥ 9 cm; less = reduced dorsiflexion
☐	Thompson / Simmonds	Patient kneels on a chair; palpate Achilles; squeeze calf just below its widest point	No plantarflexion = Achilles rupture
☐	Mulder's sign	Squeeze the metatarsals together and press up from the sole between 3rd and 4th toes, just proximal to MTPs	Pain (± click) = Morton's neuroma

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the knee, do a neurovascular exam of the foot, and assess footwear †."
☐	Present: "Normal gait. No swelling, deformity or skin changes; arches preserved. No tenderness, including over the malleoli and base of the 5th metatarsal. Full range at the ankle, subtalar and toe joints. Knee-to-wall over 9 cm; Thompson and Mulder's tests negative."

4. Quick reference †

- **Ottawa ankle rules**: X-ray if malleolar pain plus bone tenderness at the posterior edge or tip of either malleolus, or unable to weight-bear 4 steps. Foot X-ray if tender at the base of the 5th metatarsal or navicular.
- Acute hot, swollen 1st MTP = think gout; exclude septic arthritis if febrile or unwell.